

Multidisciplinary Referral Flowchart

Who to refer to · when · how to write the letter

🚑 EMERGENCY PATHWAY → ED / 000

- GCS ≤ 13 / declining
- Focal neuro deficit
- Seizure
- Repeated vomiting
- Severe / worsening HA
- Skull fracture signs
- Suspected cervical injury
- Anticoagulant + head injury
- Age > 65 + anticoagulant

TIER 1 — Primary Team (every PPCS patient)

General Practitioner

Always. Continuity, medication, mental health care plan, medical certificates, Medicare items.

Exercise Physiologist

Exertion intolerance, POTS features, return-to-sport progression. BCTT-guided rehab.

Clinical Psychologist

PHQ-9 ≥ 10 , GAD-7 ≥ 10 , trauma context, persistent symptoms > 4 weeks.

Physio / Osteo / Chiro

Cervicogenic or vestibular phenotype. Often the coordinator of care.

TIER 2 — Phenotype-Specific

Sports & Exercise Physician

> 8 wks persistent, multiple phenotypes, diagnostic uncertainty.

Neuro-Optometrist

Abnormal VOMS, NPC > 10 cm, reading intolerance despite rehab.

Occupational Therapist

Cognitive phenotype affecting work, return to driving, task modification.

Neuropsychologist

Cognitive symptoms > 4 wks. Medico-legal documentation. Baseline comparison.

Speech Pathologist

Language, word-finding, cognitive-communication difficulty.

TIER 3 — Specialist Pathways

Neurologist

Atypical / refractory / > 3 months without improvement / new features.

ENT / Vestibular Specialist

BPPV refractory to office manoeuvres. Peripheral vestibular pathology.

Endocrinologist

Persistent fatigue + weight change, amenorrhoea, libido change.

Psychiatrist

Refractory depression/anxiety, SI/HI, severe PTSD.

Pain Specialist

Headache or pain refractory to 1st/2nd-line management.

💰 PUBLIC-SYSTEM PATHWAYS

- Public hospital outpatient neurology / rehab
- Headspace (12–25) — free mental health
- GP Mental Health Care Plan — 10 psych sessions
- Medicare 10960/10961 — allied health chronic disease items
- Workers' comp / CTP (MVA) — specialist cover may be available
- ACCHS — for Aboriginal / Torres Strait Islander patients
- NDIS — only for permanent functional impact (rare)

✉ REFERRAL LETTER — 5-POINT TEMPLATE

- 1. One-line clinical framing** — “I’m writing re [name, DOB] with PPCS after [mechanism] on [date].”
- 2. Key findings** — SCOAT6 score, dominant phenotype, exam. Attach SCOAT6.
- 3. Specific request** — “Would appreciate [assessment / treatment] — particularly [clinical question].”
- 4. Patient context** — awareness, willingness, preferences/constraints.
- 5. Contact for discussion** — your phone / email. Close the loop.

🔄 CLOSED-LOOP PRINCIPLE

A referral is not complete when sent — it’s complete when the referred clinician has seen the patient, communicated back, and the plan is updated. Confirm receipt. Follow up if no response in 2–3 weeks. Read the consultation report. Integrate findings. Update the patient.

CULTURAL SAFETY

For Aboriginal and Torres Strait Islander patients: engage with Aboriginal Community Controlled Health Services where available · involve Aboriginal Health Workers · recognise family / community decision-making structures · ask the patient about preferences before referring to mainstream services.